INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Paclitaxel + cetuximab every 4 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00696
Version 2

28-day cycles: paclitaxel Days 1, 8, 15; cetuximab Days 1, 15.

Treatment stages

01 **Combination treatment**
Every 4 weeks

02 **Day 8 within the same 28-day cycle**
Paclitaxel-only visit

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shivering, sudden illness, persistent vomiting, diarrhoea, severe mouth pain or inability to drink. Report breathing difficulty, swelling, faintness, spreading skin infection or painful eyes promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

Paclitaxel and cetuximab aim to control head and neck cancer that has returned or spread and may help symptoms. Your team explains expected benefit, uncertainty and options in light of your previous treatment.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks a growth signal on cells

Cetuximab attaches to the epidermal growth factor receptor (EGFR) on cells and blocks growth signals. It is combined with chemotherapy in this plan. Healthy skin and nails also depend on this pathway, explaining some of the characteristic rash, dryness and nail problems. This is a different treatment from checkpoint immunotherapy.

EGFR blocked → Growth messages reduced → Cancer growth restrained

What to watch for

Tell staff immediately about itching, flushing, swelling, wheeze or faintness. Report skin infection, eye pain, new breathlessness or marked cramps.

TAXANE CHEMOTHERAPY

Paclitaxel

Paclitaxel



Disrupts the machinery of cell division

Paclitaxel stabilises tiny structures called microtubules so cells cannot reorganise them normally to divide. It can also affect blood-cell production, hair follicles and nerves. Tell the team about new tingling or loss of function before another dose; nerve symptoms may build up and can persist.

Microtubules stabilised → Cell division interrupted → Nerves can also be affected

What to watch for

Report fever, new numbness or weakness, difficulty walking or using your hands, and symptoms during the infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep the pathology, stage and agreed treatment aim with your records.

Site and type

Ask which head or neck site the cancer started in, what the biopsy shows, how far it has spread and how previous treatment affects the options. HPV/p16 tests apply to some tumour sites; they are different from tests of chemotherapy safety.

Who this plan is for

Treatment of recurrent or metastatic squamous cell cancer of the head and neck.

Cancer tests and treatment-safety tests

Blood counts and organ tests support safe treatment. Scans and examinations assess cancer response. These are separate questions; ask what each planned test is intended to show.

Understanding the evidence

A phase II study evaluated paclitaxel with cetuximab every 2 weeks after previous platinum chemotherapy and anti-PD-1 treatment.

This provides evidence of activity in a selected previously treated population. The study used paclitaxel 100 mg/m², whereas this NCCP schedule lists 80 mg/m² with an optional consultant-selected 100 mg/m² start. Its group results cannot be treated as a prediction for the default dose or every patient. Population: Patients with recurrent or metastatic head and neck cancer previously treated with both platinum and anti-PD-1 therapy. Comparison: A single treatment group; there was no randomised comparison arm. Small single-arm study; no proof of superiority over another treatment. Previous treatments and paclitaxel starting dose affect how the findings apply. The combination indication and schedule require off-label discussion under NCCP.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment stages and visit types

Treatment stages

01 **Combination treatment**
Every 4 weeks

02 **Day 8 within the same 28-day cycle**
Paclitaxel-only visit

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 4 WEEKS

Combination treatment

28-day cycles: paclitaxel Days 1, 8, 15; cetuximab Days 1, 15.

Paclitaxel is given on Days 1, 8 and 15, with cetuximab on Days 1 and 15 of each 28-day cycle. There is no scheduled treatment dose on Day 22. Both medicines continue while appropriate, until progression or unacceptable side effects; the protocol does not set a fixed total number of cycles.

PACLITAXEL-ONLY VISIT

Day 8 within the same 28-day cycle

Day 8; no cetuximab dose scheduled

Day 8 is a paclitaxel-only visit, with its own safety review and premedication plan. It does not start a separate course.

Visits within a cycle are not separate courses. The team records delays, omitted doses and any change of plan. A stamp records completed treatment; it does not authorise the next dose.

BASELINE → TREATMENT → FOLLOW-UP

Your treatment, tests and checks

Required regimen tests, symptom-triggered investigations and cancer-response tests serve different purposes. Ask who books each test and who explains the result.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00696 v2: official regimen · EMA: Erbitux product information and patient leaflet

Tests in your plan

Full blood count

WHEN
At baseline and regularly; your team specifies visit-by-visit timing

Checks infection-fighting cells, oxygen-carrying cells and platelets.

Kidney and liver tests

WHEN

At baseline and regularly; your team specifies visit-by-visit timing

Guide safe treatment and identify organ problems.

Electrolytes, including magnesium

WHEN

Before and periodically during cetuximab under product information; no exact universal interval is stated in this NCCP protocol

Cetuximab can lower minerals; the team agrees which tests and replacement are needed.

Imaging and tumour-specific tests

If part of your plan

WHEN

As individually arranged for the disease and symptoms; no universal marker panel or imaging interval

Assesses the cancer rather than whether a treatment-day blood count is safe.

THE WHOLE TREATMENT JOURNEY

Paclitaxel Days 1, 8, 15 + cetuximab Days 1, 15 · 28-day cycle: treatment and checks

Recurrent and/or metastatic head and neck squamous cell carcinoma under NCCP 00696.

Paclitaxel is given on Days 1, 8 and 15, with cetuximab on Days 1 and 15 of each 28-day cycle. There is no scheduled treatment dose on Day 22. Both medicines continue while appropriate, until progression or unacceptable side effects; the protocol does not set a fixed total number of cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests; assessment of nerve symptoms when indicated. Review previous reactions to antibody infusions, red-meat allergy, tick bites and any cetuximab allergy-test results, as well as all medicines and allergies.	Regular full blood count, kidney and liver tests, with nerve assessment as indicated. The NCCP source does not give one universal testing interval: your unit must specify tests due before each planned visit. Cetuximab product information also requires electrolyte assessment before and periodically during treatment; the team confirms magnesium and other mineral checks.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Continue review of nerve symptoms, skin or nail problems and any abnormal blood or mineral results. The team sets the follow-up tests and disease assessments; this regimen does not mandate a universal post-treatment scan or tumour-marker timetable.

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment and appointments NCCP	Confirm the diagnosis, aim, prescription and actual calendar.	Cetuximab 500 mg/m² is infused on Days 1 and 15. The initial infusion has a slower maximum rate than later tolerated doses; total time depends on the prescribed dose. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit. Paclitaxel is usually 80 mg/m² over 1 hour on Days 1, 8 and 15; a consultant may select a 100 mg/m² starting dose for some patients. The actual prescribed dose and premedicines must be confirmed. Day 8 is paclitaxel only.	Follow the written home plan and keep pump or line appointments where applicable.	Paclitaxel is given on Days 1, 8 and 15, with cetuximab on Days 1 and 15 of each 28-day cycle. There is no scheduled treatment dose on Day 22. Both medicines continue while appropriate, until progression or unacceptable side effects; the protocol does not set a fixed total number of cycles.	Record the last dose and follow-up appointments.
Mouth, weight and swallowing NCCP	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Cancer assessment NCCP	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

What each check is for

1 **Combination treatment**

Follow your agreed treatment calendar

Paclitaxel and cetuximab aim to control head and neck cancer that has returned or spread and may help symptoms. Your team explains expected benefit, uncertainty and options in light of your previous treatment.

Ask / **act:** Confirm active medicines, written support plan and review criteria.

2 **Day 8 within the same 28-day cycle**

Follow your agreed treatment calendar

Paclitaxel and cetuximab aim to control head and neck cancer that has returned or spread and may help symptoms. Your team explains expected benefit, uncertainty and options in light of your previous treatment.

Ask / **act:** Confirm active medicines, written support plan and review criteria.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Bring your medicines and allergy list, test dates, treatment calendar and actual emergency contacts. Tell the unit about swallowing, hearing, mobility, transport or communication needs.

STEP 2

Time in the unit

Cetuximab 500 mg/m² is infused on Days 1 and 15. The initial infusion has a slower maximum rate than later tolerated doses; total time depends on the prescribed dose. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit. Paclitaxel is usually 80 mg/m² over 1 hour on Days 1, 8 and 15; a consultant may select a 100 mg/m² starting dose for some patients. The actual prescribed dose and premedicines must be confirmed. Day 8 is paclitaxel only.

STEP 3

Infusion and appointment planning

Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit. Day 8 is a shorter paclitaxel-only visit, but checks, preparation and observation still add time. Ask the unit for a realistic arrival and collection plan.

STEP 4

Before you leave

Check the next treatment and test dates, medicines, pump or line appointments, and who to contact if unwell. Have the team replace and verify every example phone number on the wallet card.

STEP 5

Practical support

Ask about transport, work, costs, caring responsibilities, an interpreter or a support person. Keep changes to the plan in one record.

Recovery, review and longer-term care

Finishing scheduled doses and recovering are different milestones.

Symptoms and follow-up

Continue review of nerve symptoms, skin or nail problems and any abnormal blood or mineral results. The team sets the follow-up tests and disease assessments; this regimen does not mandate a universal post-treatment scan or tumour-marker timetable.

The next cancer assessment

Record who arranges examinations, scans and any tumour-specific tests, and who explains the results. A general reference calendar does not set an appointment or mean a scan is overdue.

Information for other clinicians

Keep the regimen name, last treatment date, current medicines and unresolved side effects available. Tell future clinicians about any earlier head and neck radiotherapy, platinum-related hearing or kidney problems, and significant infusion reactions.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cetuximab + paclitaxel	 **Day 1** Both infusions after the combined premedication plan.
Paclitaxel only	 **Day 8** Day 8 has its own premedication plan.
Cetuximab + paclitaxel	 **Day 15** Both infusions after safety review.
No scheduled dose	 **Days 22–28** Symptoms and blood tests may still need attention.

Treatment sequence: Paclitaxel is given on Days 1, 8 and 15, with cetuximab on Days 1 and 15 of each 28-day cycle. There is no scheduled treatment dose on Day 22. Both medicines continue while appropriate, until progression or unacceptable side effects; the protocol does not set a fixed total number of cycles.

Appointments and administration: Cetuximab 500 mg/m² is infused on Days 1 and 15. The initial infusion has a slower maximum rate than later tolerated doses; total time depends on the prescribed dose. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit. Paclitaxel is usually 80 mg/m² over 1 hour on Days 1, 8 and 15; a consultant may select a 100 mg/m² starting dose for some patients. The actual prescribed dose and premedicines must be confirmed. Day 8 is paclitaxel only.

Baseline and ongoing checks: Full blood count, kidney and liver tests; assessment of nerve symptoms when indicated. Review previous reactions to antibody infusions, red-meat allergy, tick bites and any cetuximab allergy-test results, as well as all medicines and allergies. Regular full blood count, kidney and liver tests, with nerve assessment as indicated. The NCCP source does not give one universal testing interval: your unit must specify tests due before each planned visit. Cetuximab product information also requires electrolyte assessment before and periodically during treatment; the team confirms magnesium and other mineral checks.

Recovery and follow-up: Continue review of nerve symptoms, skin or nail problems and any abnormal blood or mineral results. The team sets the follow-up tests and disease assessments; this regimen does not mandate a universal post-treatment scan or tumour-marker timetable.

Why these bands stay broad: This shows the prescribed pattern, not guaranteed symptom dates. Actual dates, delays and pump times must be confirmed by the team. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00696 v2: official regimen ↗

There are no guaranteed "safe days". Your prescription and team's instructions take priority. Contact the team about new symptoms rather than waiting for a scheduled visit.

Between-treatment notes

Day 1: Both medicines

Cetuximab and paclitaxel after safety review and the combined premedication plan. Tell staff immediately about any infusion symptoms.

Day 2–14: Between visits, with paclitaxel on Day 8

Day 8 is paclitaxel only. Tingling, aches, bowel symptoms and skin changes may develop between visits. Report symptoms before treatment and follow the correct Day 8 support plan.

Day 15: Both medicines again

Cetuximab and paclitaxel after review. Bring an updated symptom and medicine record.

Day 16–28: Recovery and tests

No dose is scheduled on Day 22. Infection, skin, nerve or other problems can still arise. Arrange the tests required for the next visit; call if unwell.

Support around the medicines

1 One written support-medicine plan

Ask for one list separating prevention medicines, rescue treatment and regular medicines. The schedule must match this treatment and visit. Record what you take and whether it helps; do not double a dose, restart an old prescription or add medicines without checking.

2 Preventing sickness

Each medicine is classified as having a low sickness risk, but symptoms still need an individual plan. Take any prescribed anti-sickness medicine as directed and keep rescue instructions available. Report persistent vomiting or difficulty drinking.

3 Cetuximab premedicines and delayed reactions

An antihistamine and corticosteroid are given before cetuximab to reduce reaction risk. They do not prevent every reaction. Report itching, flushing, swelling, wheeze, dizziness or faintness immediately, including symptoms that start after leaving the unit.

4 Skin, nails, eyes and minerals

Use the skin-care plan from the team, including moisturising and sun protection. Report painful cracks, inflamed nails, pus or spreading redness. Eye pain or visual change needs prompt review. Keep prescribed blood tests for magnesium and other minerals; supplements must follow clinical advice.

5 Different premedicines on different visits

On Days 1 and 15, the steroid and antihistamine for cetuximab also cover these parts of the paclitaxel premedication plan; staff coordinate any additional medicine. Day 8 has a separate paclitaxel-only plan. Later steroid or stomach-receptor-blocker doses may change after tolerance review; do not omit or duplicate medicines yourself.

6 Food, fluids and weight

Tell the team early about reduced intake or weight loss. A dietitian can adapt food texture, supplements and intake goals. Follow an individual fluid plan, especially with kidney or heart problems.

7 Mouth and swallowing care

Follow the mouth-care and pain-relief plan. Report ulcers, thrush or swallowing difficulty early. Ask for a swallowing assessment if food or drinks make you cough or choke; do not assume a general food texture is safe for you.

8 Fertility, pregnancy and medicine checks

Before treatment, discuss fertility, contraception, pregnancy and breastfeeding, including medicine-specific precautions after treatment. Bring all prescription, over-the-counter and herbal medicines to review. The team must check interactions and allergies.

Treatment choices and your priorities

The consent discussion sets out the personal aim and boundaries.

Expected benefit and alternatives

Paclitaxel and cetuximab aim to control head and neck cancer that has returned or spread and may help symptoms. Your team explains expected benefit, uncertainty and options in light of your previous treatment. Ask about reasonable alternatives, including supportive care, and which outcomes matter most to you. A research result cannot predict an individual outcome.

Changing or stopping treatment

The team may delay, reduce, omit or stop a medicine after symptoms or test results, a change in benefit or your preference. Record which medicine is affected and what is required before restarting. Do not use an old prescription to restart treatment.

Other treatment and off-label use

NCCP identifies this paclitaxel–cetuximab combination and schedule as off-label use; the team must confirm the current licensing position for your actual prescription. Discuss the reasons, evidence, alternatives and uncertainties and document the local consent process. Surgery or radiotherapy may have a separate role, but they are not automatically added to this infusion calendar.

Preparing for your treatment visits

Use the medicine schedule and logistics pages for the exact visit pattern.

Your personal checklist

Check that the treatment name, planned tests, support medicines, transport and urgent contacts are correct. Explain any previous infusion reaction or change in your health before the medicine is prepared.

When dates change

A missed or held dose is not a completed dose. Ask for revised blood-test, treatment and pump dates where relevant. The team must clarify whether an omitted weekly dose changes later dates.

Recording actual treatment

Record the actual date and medicines received, dose changes, test results and symptoms. The wallet is a brief portable record; the passport holds more detail and does not replace hospital records.

Between visits: symptoms and home medicines

Symptoms do not follow a guaranteed calendar.



One usable medicine list

Keep prevention, rescue and regular medicines separately labelled. Note doses actually taken and whether symptoms improve. Check new prescription, over-the-counter and herbal medicines with the team; do not add or repeat old courses without advice.

Nerve symptoms and daily function

Report numbness, tingling, weakness, falls, difficulty doing buttons or changes in walking. Describe what you can no longer do, not just a symptom score. The team may need to change treatment before symptoms worsen.

Mouth, teeth and jaw care

This is the home for detailed mouth and dental advice.

Care during chemotherapy

Use the prescribed gentle oral-care and pain-relief plan. Report ulcers, infection, bleeding or pain that affects eating or drinking. Check with oncology before dental procedures.

If you have had head and neck radiation

If you have had head and neck radiation, tell your dentist before invasive treatment. Continue the advised preventive care because effects on saliva, teeth and healing can persist. If you have not had radiation, follow the plan for your current treatment.

Eating, drinking and maintaining strength

Adapt the plan to what you can manage safely.

Weight and intake

Record changes in weight and what limits eating: soreness, taste change, dry mouth, nausea or fatigue may need different help. A dietitian can adapt meals, supplements or feeding support. Do not wait for substantial weight loss to ask for review.

When fluids or food are not enough

Contact the team for persistent vomiting, dizziness, much less urine, inability to drink or severe diarrhoea. Fluid goals depend on kidney and heart health. If a feeding tube is needed, ask for training, supplies and a clear problem-contact route.

Swallowing, voice and communication

Protect safe swallowing and make communication easier.

When to seek assessment

Report coughing or choking with food or drinks, a wet voice after swallowing, food sticking or repeated chest infections. Ask for speech and language assessment; use only the textures and exercises recommended for you.

Communication support

Tell the team about hearing, speech or voice changes. Request written instructions, an interpreter, communication aids or help involving a support person when useful.

Daily life and personal priorities

Use the support available around you.

Activity, rest and practical needs

Discuss the level of activity you can manage, work, travel and help at home. Ask for assessment if fatigue, pain, weakness or falls disrupt ordinary activities. Bring practical barriers to appointments into the discussion.

Fertility, intimacy and emotional wellbeing

Ask about fertility preservation before treatment and specific pregnancy, contraception and breastfeeding precautions. Tell the team about anxiety, body-image, sexual or relationship concerns; support can be tailored to your priorities.

Smoking and alcohol

Ask for help with smoking cessation and discuss alcohol with the team. Mouth symptoms, medicines and recovery may affect what is appropriate for you.

Your records and additional help

Essential information should remain usable on paper.

Passport and treatment card

Use the passport for the fuller record and the wallet card for the treatment name, contacts and actual completed cycles. Its QR opens this treatment's information; it does not contain a personal medical record or update paper stamps.

Complete local contacts

Every example telephone number must be replaced and checked by the treating service before patient use. Add daytime, out-of-hours and backup contacts and keep them accessible to a support person.

Reading without a phone

Read the full handbook in its chapter sequence. The treatment, symptom, contact and practical information does not require scanning a QR or following a hyperlink. Add questions to the passport and bring it to review.

Questions worth asking

What is the aim of this treatment for me, and what are the alternatives?

Paclitaxel and cetuximab aim to control head and neck cancer that has returned or spread and may help symptoms. Your team explains expected benefit, uncertainty and options in light of your previous treatment. Ask your team to write down the individual aim and explain reasonable alternatives, their burdens and the uncertainties before you decide.

Which medicines and tests are due at each visit?

Paclitaxel is given on Days 1, 8 and 15, with cetuximab on Days 1 and 15 of each 28-day cycle. There is no scheduled treatment dose on Day 22. Both medicines continue while appropriate, until progression or unacceptable side effects; the protocol does not set a fixed total number of cycles. Regular full blood count, kidney and liver tests, with nerve assessment as indicated. The NCCP source does not give one universal testing interval: your unit must specify tests due before each planned visit. Cetuximab product information also requires electrolyte assessment before and periodically during treatment; the team confirms magnesium and other mineral checks. Confirm the actual dates with your unit.

Which symptoms mean I should call immediately?

Call oncology immediately for fever, shivering, sudden illness, persistent vomiting, diarrhoea, severe mouth pain or inability to drink. Report breathing difficulty, swelling, faintness, spreading skin infection or painful eyes promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency. Have the team replace the example numbers with verified daytime, out-of-hours and backup contacts before using the card.

How will we decide whether to continue, change or stop?

Your team reviews symptoms, test results, treatment benefit and your priorities. Ask when cancer-response assessment is planned and what would lead to a change. A delay or omitted dose must be recorded and does not count as treatment received.

What support can help me manage treatment?

Ask for a written support-medicine plan and early help with mouth pain, eating, drinking, swallowing, skin or nerve symptoms. Tell the team about travel, work, caring and communication needs. Dietitian, speech and language, nursing and other support can be arranged according to your needs.

> **Try explaining the plan back:** Explain which medicines you receive on each visit. Name the symptoms that mean you should contact oncology immediately. Show the actual contact numbers and say which tests and appointments are next.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1 and 15 · cetuximab followed by paclitaxel

Active medicines: Cetuximab + Paclitaxel

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before cetuximab NCCP Table 6; this antihistamine also covers paclitaxel on combined-treatment days. Do not duplicate it.

Medicine	Dose / route	When
Dexamethasone	8 mg IV	60 minutes before cetuximab NCCP Table 6; this steroid also covers paclitaxel on Days 1/15. Reconcile total steroid dose and diabetes/sleep advice.
Famotidine	20 mg IV	30 minutes before paclitaxel NCCP Table 7. Consultant may omit after review; the source notes uncertainty about hypersensitivity risk when omitted.
Low-risk / rescue antiemetic	Individual prescription	If required after accounting for infusion steroid No routine moderate-risk course. Persistent symptoms warrant review.

Additional phase-specific support

- Give an antihistamine and a corticosteroid at least one hour before cetuximab; continue this premedication at subsequent doses. Reconcile steroids with any antiemetic prescription to avoid duplication.
- Ask specifically about earlier antibody reactions, red-meat allergy, tick bites and any cetuximab IgE result. Premedication does not eliminate severe or delayed reactions.
- Use a local EGFR-inhibitor skin plan, moisturiser and sun protection; assess painful, infected or intolerable rash and nail changes promptly.
- NCCP allows observation to cease after two reaction-free cetuximab doses, whereas current Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Agree and document the governed observation plan; do not automatically remove observation.
- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- Consider prescribed analgesia for paclitaxel-related muscle/joint pain. Assess neuropathy before it becomes disabling; do not wait for a routine cycle boundary.
- Both medicines are low emetic risk in NCCP v2; do not carry forward an unsupported moderate-risk antiemetic schedule. Day 1/15 cetuximab steroid/antihistamine premedication also covers paclitaxel; Day 8 needs its distinct plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · paclitaxel only

Active medicines: Paclitaxel

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	30 minutes before paclitaxel NCCP Table 7.
Famotidine	20 mg IV	30 minutes before paclitaxel May be omitted only according to consultant guidance and reaction history.
Dexamethasone · conditional	No routine Day 8 dose in NCCP Table 7	Add only if increased risk or previous hypersensitivity, under consultant guidance This differs from the steroid required before cetuximab on Days 1/15. A rechallenge steroid schedule is a separate specialist decision.
Low-risk antiemetic reference	Ondansetron 8 mg orally once on the treatment day if selected	Before treatment under the agreed plan NCIS low-risk reference; individualise QT risk and other medicines. Any rescue medicine and its duration must be prescribed.

Additional phase-specific support

- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- Review neuropathy and previous paclitaxel reactions. Day 8 is a paclitaxel-only visit within the same 28-day cycle, not maintenance or a different course.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hands / feet / nerves

Paclitaxel

Report numbness, tingling, weakness or difficulty walking or using your hands before another dose.

3 Mouth / bowels

Paclitaxel + cetuximab

Report mouth sores, diarrhoea, pain or reduced drinking; ask for help before dehydration develops.

5 Skin / nails / hair

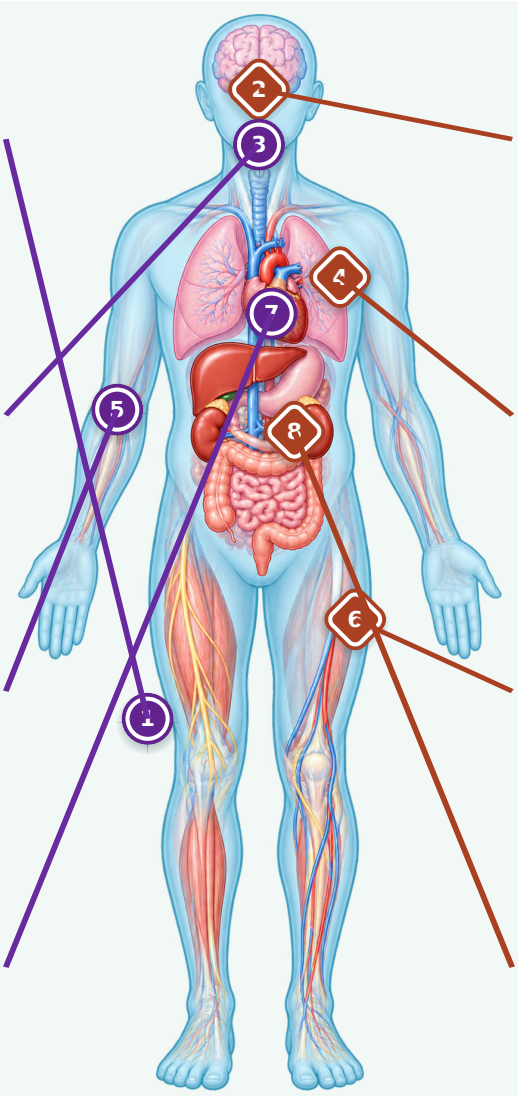
Cetuximab + paclitaxel

Rash, cracked skin and nail infection need care; paclitaxel can cause hair loss. Report spreading redness or fever.

7 Blood cells / infection

Chemotherapy

Fever, shivering, sudden illness, unusual bruising or bleeding needs prompt oncology advice.



2 Eyes

Cetuximab

Report painful or red eyes, light sensitivity or a change in vision promptly.

4 Lungs / breathing

Cetuximab

Report new cough or breathlessness. Wheeze, swelling or faintness may be an infusion reaction.

6 Muscles / joints

Paclitaxel

Aches may occur after treatment. Report severe pain or weakness; follow the agreed pain-relief plan.

8 Minerals / fluids

Cetuximab

Low magnesium or other minerals may show on blood tests. Report cramps, weakness or palpitations.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shivering, sudden illness, persistent vomiting, diarrhoea, severe mouth pain or inability to drink. Report breathing difficulty, swelling, faintness, spreading skin infection or painful eyes promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00696 v2 · pNCCP 00696: treatment and supportive-care sections; NCIS v8 pp4–5 where used**
   https://healthservice.hse.ie/documents/6769/696_PACLitaxel_Cetuximab_28day.pdf
2. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
3. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
4. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
5. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
6. **Koyama 2024: paclitaxel with biweekly cetuximab, phase II study**
   https://pubmed.ncbi.nlm.nih.gov/38833968/
7. **NCCP 00696 and current NCIS/local supportive-care guidance**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf